

Hospital Discharge Summary

Patient: Robert Alvarez

Admission date: August 1, 2026 Discharge date: August 5, 2026

Service: Cardiology

Diagnosis: Acute decompensated heart failure, resolved with intravenous diuresis.

Hospital course: Mr. Alvarez was admitted with acute decompensated heart failure and treated with intravenous Furosemide, transitioning to an oral dose of 40 mg twice daily prior to discharge. Lisinopril 5 mg oral once daily was started for afterload reduction and tolerated well, with stable renal function and potassium. Metoprolol tartrate was discontinued and switched to metoprolol succinate 50 mg oral once daily for improved once-daily dosing. Amlodipine was discontinued given its likely contribution to the peripheral edema on presentation. He diuresed well, returning to his dry weight, and was ambulating without dyspnea at discharge.

Discharge medications:

- Furosemide 40 mg oral twice daily
- Lisinopril 5 mg oral once daily
- Metoprolol succinate 50 mg oral once daily
- Atorvastatin 20 mg oral nightly
- Aspirin 81 mg oral once daily

Discontinued during this admission:

- Amlodipine 10 mg oral once daily - discontinued (peripheral edema)
- Metoprolol tartrate 25 mg oral twice daily - discontinued (replaced by succinate formulation)

Follow-up: Cardiology clinic in one week. Basic metabolic panel to recheck renal function and potassium at that visit.